**DIAGNOSTIC REPORT**

MC-5718

agilus diagnostics | Formerly SRL
Dr. Phadke Labs

**PATIENT NAME : HEMA GANDHI** | **REF. DOCTOR :** DR. MANOJ MASHROO

HEMA GANDHI
501 B WING,, VASTU RIDDHI CHS, JIJAMATA ROAD, NEAR MANISH PARK, OPP PARSI COLONY,PUMP
400059

ACCESSION NO : **0065XA000809**
PATIENT ID : HEMAF2103570
CLIENT PATIENT ID: HEMAF2103570
ABHA NO :

AGE/SEX : 67 Years Female
DRAWN : 11/01/2024 15:00:00
RECEIVED : 11/01/2024 15:43:29
REPORTED : 11/01/2024 20:46:09

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## HAEMATOLOGY - CBC

**CBC-5, EDTA WHOLE BLOOD**

**BLOOD COUNTS, EDTA WHOLE BLOOD**

| | | | |
|---|---|---|---|
| HEMOGLOBIN (HB)<br>METHOD : CYANIDE FREE DETERMINATION | 12.8 | 12.0 - 15.0 | g/dL |
| RED BLOOD CELL (RBC) COUNT<br>METHOD : ELECTRICAL IMPEDANCE | 4.15 | 3.8 - 4.8 | mil/µL |
| WHITE BLOOD CELL (WBC) COUNT<br>METHOD : FLUORESCENCE FLOW CYTOMETRY | 4.87 | 4.0 - 10.0 | thou/µL |
| PLATELET COUNT<br>METHOD : ELECTRONIC IMPEDENCE & MICROSCOPY | **143 Low** | 150 - 410 | thou/µL |

**RBC AND PLATELET INDICES**

| | | | |
|---|---|---|---|
| HEMATOCRIT (PCV)<br>METHOD : CALCULATED PARAMETER | 39.1 | 36 - 46 | % |
| MEAN CORPUSCULAR VOLUME (MCV)<br>METHOD : DERIVED PARAMETER FROM RBC HISTOGRAM | 94.3 | 83 - 101 | fL |
| MEAN CORPUSCULAR HEMOGLOBIN (MCH)<br>METHOD : CALCULATED PARAMETER | 30.8 | 27.0 - 32.0 | pg |
| MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION(MCHC)<br>METHOD : CALCULATED PARAMETER | 32.7 | 31.5 - 34.5 | g/dL |
| RED CELL DISTRIBUTION WIDTH (RDW)<br>METHOD : DERIVED PARAMETER FROM RBC HISTOGRAM | **15.0 High** | 11.6 - 14.0 | % |
| MENTZER INDEX<br>METHOD : CALCULATED PARAMETER | 22.7 | | |
| MEAN PLATELET VOLUME (MPV)<br>METHOD : DERIVED PARAMETER FROM PLATELET HISTOGRAM | **12.9 High** | 6.8 - 10.9 | fL |

**WBC DIFFERENTIAL COUNT**

| | | | |
|---|---|---|---|
| NEUTROPHILS<br>METHOD : FLUORESCENCE FLOW CYTOMETRY | 66 | 40 - 80 | % |
| LYMPHOCYTES<br>METHOD : FLUORESCENCE FLOW CYTOMETRY | **18 Low** | 20 - 40 | % |

**Dr. Sushant Chikane**
**Consultant Pathologist**

**Dr. Akshaya Mandloi,MD,PDF**
**Consultant Hematopathologist and Head,**
**Dept.of Hematopathology and Flowcytometry**



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Mumbai, 400062
Maharashtra, India
Tel : 67801243 / 67801244, Fax :
CIN - U74899PB1995PLC045956

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| MONOCYTES<br>METHOD : FLUORESCENCE FLOW CYTOMETRY | 7 | 2 - 10 | % |
| EOSINOPHILS<br>METHOD : FLUORESCENCE FLOW CYTOMETRY | **8 High** | 1 - 6 | % |
| BASOPHILS<br>METHOD : FLUORESCENCE FLOW CYTOMETRY | 1 | 0 - 2 | % |
| ABSOLUTE NEUTROPHIL COUNT<br>METHOD : FLOUROSCENT FLOW CYTOMETRY / MICROSCOPY, CALCULATED | 3.21 | 2.0 - 7.0 | thou/µL |
| ABSOLUTE LYMPHOCYTE COUNT<br>METHOD : CALCULATED PARAMETER | **0.88 Low** | 1.0 - 3.0 | thou/µL |
| ABSOLUTE MONOCYTE COUNT<br>METHOD : CALCULATED PARAMETER | 0.34 | 0.2 - 1.0 | thou/µL |
| ABSOLUTE EOSINOPHIL COUNT<br>METHOD : FLOUROSCENT FLOW CYTOMETRY / MICROSCOPY, CALCULATED | 0.39 | 0.02 - 0.50 | thou/µL |
| ABSOLUTE BASOPHIL COUNT<br>METHOD : CALCULATED PARAMETER | 0.05 | 0.02 - 0.10 | thou/µL |
| NEUTROPHIL LYMPHOCYTE RATIO (NLR)<br>METHOD : CALCULATED | 3.6 | | |

**MORPHOLOGY**

| | |
|---|---|
| RBC | PREDOMINANTLY NORMOCYTIC NORMOCHROMIC |
| WBC | NORMAL MORPHOLOGY |
| PLATELETS | REDUCED |

**Interpretation(s)**
RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate cases of Iron deficiency anaemia(>13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and suspicion. Estimation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.
WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive patients. When age = 49.5 years old and NLR = 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease. (Reference to - The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients A.-P. Yang, et al. International Immunopharmacology 84 (2020) 106504
This ratio element is a calculated parameter and out of NABL scope.

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| **COAGULATION** | | | |
| **PROTHROMBIN TIME, CITRATE PLASMA** | | | |
| PROTHROMBIN TIME (PT)<br>METHOD : CLOT BASED ASSAY | **54.5 High** | 12.0 - 14.0 | SECONDS |
| INTERNATIONAL NORMALIZED RATIO (INR) | **4.60 C.High** | 0.80 - 1.10 | RATIO |
| MEAN PROTHROMBIN TIME OF CONTROL PLASMA (MNPT) | 13.1 | | SECONDS |

**Comments**

Advised :
1) Kindly correlate with clinical and treatment history.
2) Kindly correlate with clinically with the oral anticoagulation therapy and possible drug interaction.

**Interpretation(s)**
PROTHROMBIN TIME, CITRATE PLASMA-**TEST DESCRIPTION**- Prothrombin Time measures the integrity of the extrinsic pathway and the adequacy of critical coagulation factors involved in it, namely, Factor VII. This test is therefore, used for monitoring oral anticoagulation therapy which lowers the levels of multiple vitamin K dependent coagulation factors in blood (Factors II, VII, IX and X) including Factor VII. The result of PT is expressed as International Normalized Ratio (INR) to neutralize the influence of variable sensitivity of the reagents (thromboplastin) used in the assay by different laboratories
**TEST INTERPRETATION- INCREASED PT** may be due to:
1.Factor deficiencies,2. Drugs (e.g. Coumarin-type drugs for anticoagulant therapy, salicytes),3. Severe liver damage (e.g. poisoning, hepatitis, cirrhosis),4.Hypofibrinogenemia (Acquired or Inherited),5.Hemorrhagic disease of the newborn.,6 Poor fat absorption (e.g.obstructive jaundice,fistulas,sprue,steatorrhea,celiac disease,colitis,chronic diarrhoea.)
**RECOMMENDATION-This is a very sensitive reagent and therefore it is advisable of follow up I.N.R. value rather than P.T. in seconds.The recommended I.N.R. is 2 - 3 for patients on oral anticoagulant in all conditions except mechanical valve replacement and prevention of myocardial infarction where the I.N.R. may be maintained between 2.5-3.5. Please stop anticoagulant therapy if the I.N.R. is > 4.5.**

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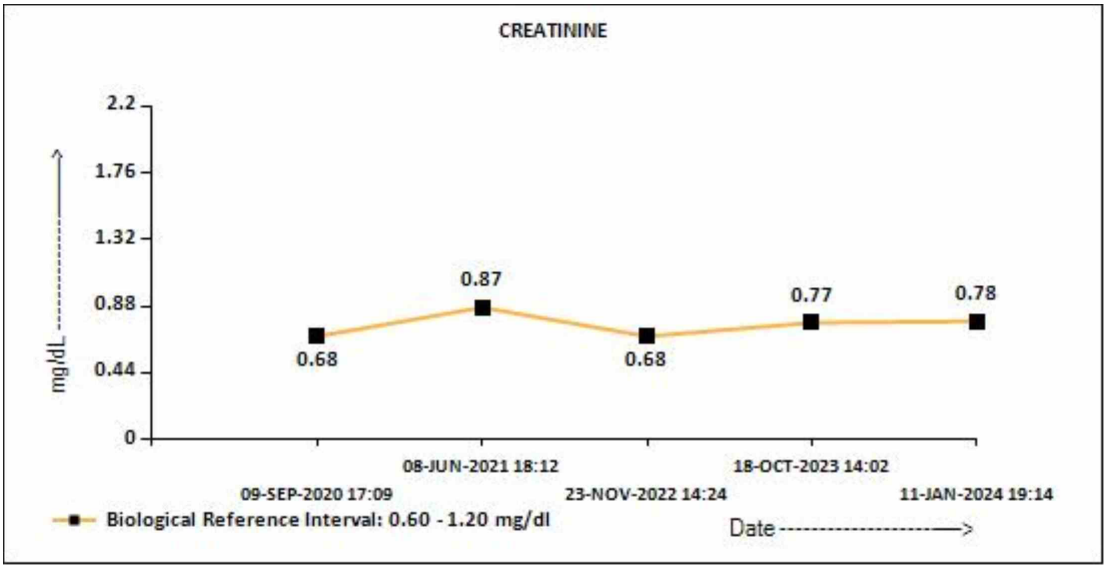
## BIOCHEMISTRY

### CREATININE, SERUM

| | | | |
|---|---|---|---|
| CREATININE | 0.78 | 0.60 - 1.20 | mg/dL |

METHOD : SPECTROPHOTOMETRY, JAFFE'S ALKALINE PICRATE KINETIC - RATE BLANKED - IFCC-IDMS STANDARIZED




### ELECTROLYTES (NA/K/CL), SERUM

| | | | |
|---|---|---|---|
| SODIUM, SERUM | 141 | 136 - 145 | mmol/L |

METHOD : ISE INDIRECT

| | | | |
|---|---|---|---|
| POTASSIUM, SERUM | 3.80 | 3.5 - 5.1 | mmol/L |

METHOD : ISE INDIRECT

| | | | |
|---|---|---|---|
| CHLORIDE, SERUM | 105 | 98 - 106 | mmol/L |

METHOD : ISE INDIRECT

**Interpretation(s)**
CREATININE, SERUM-**Higher than normal level may be due to:**
• Blockage in the urinary tract, Kidney problems, such as kidney damage or failure, infection, or reduced blood flow, Loss of body fluid (dehydration), Muscle problems, such

**Dr. Apeksha Sharma**
**D.P.B.,DNB (PATH)**
**(Reg.no.MMC2008/06/2561)**
**Consultant Pathologist**

**Dr. Deepak Sanghavi**
**Chief Of Lab - Mumbai Reference Lab**



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as breakdown of muscle fibers, Problems during pregnancy, such as seizures (eclampsia)), or high blood pressure caused by pregnancy (preeclampsia)
**Lower than normal level may be due to:•** Myasthenia Gravis, Muscuophy

****End Of Report****
**Please visit www.agilusdiagnostics.com for related Test Information for this accession**

**CONDITIONS OF LABORATORY TESTING & REPORTING**

1. It is presumed that the test sample belongs to the patient named or identified in the test requisition form.
2. All tests are performed and reported as per the turnaround time stated in the AGILUS Directory of Services.
3. Result delays could occur due to unforeseen circumstances such as non-availability of kits / equipment breakdown / natural calamities / technical downtime or any other unforeseen event.
4. A requested test might not be performed if:
   i. Specimen received is insufficient or inappropriate
   ii. Specimen quality is unsatisfactory
   iii. Incorrect specimen type
   iv. Discrepancy between identification on specimen container label and test requisition form
5. AGILUS Diagnostics confirms that all tests have been performed or assayed with highest quality standards, clinical safety & technical integrity.
6. Laboratory results should not be interpreted in isolation; it must be correlated with clinical information and be interpreted by registered medical practitioners only to determine final diagnosis.
7. Test results may vary based on time of collection, physiological condition of the patient, current medication or nutritional and dietary changes. Please consult your doctor or call us for any clarification.
8. Test results cannot be used for Medico legal purposes.
9. In case of queries please call customer care (91115 91115) within 48 hours of the report.

**Agilus Diagnostics Ltd**
Fortis Hospital, Sector 62, Phase VIII,
Mohali 160062

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